

• PARTNERSHIP PROPOSAL · VOLUME I OF V

A passive-income engine for the **Leverage alumni** community.

Halcyon Medical Billing manages the full revenue cycle for independent US medical practices. We pay **25% of the first month + 10% recurring** on every practice a partner's referral converts into a Halcyon client — for the lifetime of that relationship. We'd like to offer that economic flow to the Leverage Edu alumni and Work-Global community.

\$14.9B

US MEDICAL BILLING MARKET

155K+

LEVERAGE ALUMNI REACH

10%

RECURRING COMMISSION · NO CAP

90D

FROM SIGNATURE TO FIRST CHEQUE

WHY THIS. WHY NOW.

Three things we noticed about Leverage in the last 90 days.

Before we propose anything: we did our homework. The pitch only makes sense if it lines up with where Leverage Edu is already going.

SIGNAL · 01

The TNSDC nursing pipeline is live.

Leverage Careers' MoU with the Tamil Nadu Skill Development Corporation is now training 120+ nurses/year for global placement. Many of those nurses will land inside US independent practices — the exact buyers Halcyon serves.

SIGNAL · 02

EBITDA-positive. IPO bankers tapped.

₹375 Cr in FY26, EBITDA-positive, ₹2,000–3,000 Cr IPO in 12–18 months. The story investors will pay a premium for is **recurring, non-counselling revenue** — which is exactly the line a Halcyon partnership creates.

SIGNAL · 03

The affiliate engine already exists.

Leverage already runs a partner dashboard with UTM tracking, monthly payouts, and a 1-1-1 commission guarantee. Adding Halcyon as a second economic flow on top of that infrastructure is configuration — not a build.

The point of this proposal: Halcyon's commissions are higher per-referral than Leverage's current ₹500 / ₹20,000 student affiliate payouts — and they recur monthly, indefinitely, with no clawback. We think it slots cleanly into the IPO narrative as a high-margin, high-LTV revenue stream that compounds with every alumnus you place.



WHAT HALCYON IS

A HIPAA-compliant revenue cycle for **independent** US medical practices.

US doctors lose 10–15% of earned revenue to denied claims, slow payments, and broken billing. Halcyon runs the full cycle — eligibility, coding, claim submission, denial work, payment posting, patient collections — on a real-time client portal. Every PHI field is encrypted. Every action is logged in an append-only audit trail. The same standard hospitals use.

Registered LLP • BAA-executed stack

Halcyon Medical Billing LLP is a fully registered partnership entity with executed Business Associate Agreements across every vendor in our stack. HITECH technical safeguards in place.

Real-time client portal

Practices see every claim, payment, and denial as it happens. No black-box quarterly reports. The portal is a competitive moat against legacy billing companies.

Direct leadership access

No call center. Every onboarding, demo, and escalation is handled by the founding team. Practices keep our direct lines.

96%

TARGET FIRST-PASS ACCEPTANCE

23%

RISE IN US INSURANCE DENIALS,
2024

\$44B

MARKET SIZE BY 2034 (PROJECTED)

72%

DEALS WON THROUGH REFERRALS

THE THESIS

Your alumni already work where our buyers are.

Leverage Edu has placed **155,000+ students** across nine years and is now actively training Indian RNs, NPs, and caregivers for the US market. Indian-Americans are the single largest immigrant physician group in the US, and a measurable cohort of your alumni are now embedded inside US independent practices — as nurses, residents, partners, and practice owners.

Halcyon's buyer is the practice owner or practice manager at a 1–10-provider US clinic. Our highest-converting channel is warm introduction — **72% of medical billing contracts** in the US are won through referrals. A nurse who has worked in that clinic for two years is the exact warm introduction we'd otherwise pay \$300–\$800 to generate cold.

The proposal is simple: **open a Halcyon track inside the Leverage partner dashboard.** Alumni who have landed in the US opt in, send a one-line introduction to their practice owner, and earn ongoing income on top of their salary.

WHY IT CONVERTS

Warm intros from staff close at 8–12x.

A cold call to a US practice manager closes ~2%. A warm intro from someone they trust closes 18–25%. The economics of paying 35% gross commission only make sense when conversion is high — and it is.

WHY IT'S DEFENSIBLE

Alumni hold the relationship, not Halcyon.

A competitor can't poach this channel. The trust lives in the alumnus — and the alumnus's payout depends on the practice staying with Halcyon for the long run.

WHY IT'S IPO-FRIENDLY

Recurring, non-counselling revenue.

Halcyon commissions hit Leverage's books monthly, with no marginal cost — the alumnus does the work. Every signed practice compounds the line indefinitely.

THE HONEST MATH

Per practice that signs & stays.

No upfront, no retainer. **25%** of the practice's first-month Halcyon fee, paid once. **10%** of every subsequent monthly fee, recurring as long as the practice stays. Math:

PRACTICE SIZE	HALCYON FEE / MO	MONTH-1 COMMISSION	RECURRING / MO	YEAR 1 / PRACTICE	YEAR 2 / PRACTICE
Small primary care \$75K/mo collections	\$3,750	\$938	\$375	\$5,063	\$4,500
Mid-range \$130K/mo collections	\$6,500	\$1,625	\$650	\$8,775	\$7,800
Multi-specialty \$250K/mo collections	\$12,500	\$3,125	\$1,250	\$16,875	\$15,000

Channel revenue = closes × per-practice revenue above. We're not projecting close counts. A "close" means a signed Halcyon MSA + first invoice paid; commission flows only from there. Halcyon eats 100% of the cost of getting from intro to signature.

WHAT WE'RE ASKING

A 25-minute call. Then a 90-day pilot.

We're not asking Leverage to build anything new. We're asking for one conversation to validate fit, then a pilot inside an existing alumni segment of your choosing — no exclusivity, no upfront commitment, full clawback rights.

STEP · 01

25-minute fit call

Halcyon's CEO and CFO walk Leverage's partnerships lead through the technical onboarding, BAA structure, and dashboard integration. We answer questions. No pitch deck.

STEP · 02

90-day pilot

Open a Halcyon channel inside the existing partner dashboard. Soft-launch to a 200-alumnus segment of your choosing. Measure conversions, payouts, NPS. Decide if we scale.

STEP · 03

Scale or sunset

If pilot KPIs land, full alumni rollout with co-branded comms and a Leverage Careers cross-sell into employer accounts. If they don't, we sunset cleanly — no commitments owed.

REPLY WITH A 25-MIN SLOT ANY WEEKDAY BEFORE 27 MAY · NAITIK@HALCYONMEDBILLING.COM

WHO YOU'D BE WORKING WITH

A senior team. No middle layer.

Halcyon is run by three founders. Every partnership conversation, contract, and onboarding runs directly through us — not through an account manager or a BD generalist.

CEO & CTO

Naitik Joshi · SENDER

Architected Halcyon's HIPAA portal, encryption layer, and the 12-step RCM workflow every client runs on. Leads sales conversations and demos directly.

COO · OPERATIONS

Ankit Chauhan

Runs day-to-day billing operations and client onboarding. Manages the specialist team ensuring claims are worked correctly and on schedule.

CFO · FINANCE & PARTNERSHIPS

Rushan Khan

Owns commission disbursements and partner agreements. Your direct counterpart for this proposal — and the person who will sign the referral agreement on Halcyon's side.

COMPLIANCE & TRUST

- Halcyon Medical Billing LLP — registered, in good standing
- HIPAA & HITECH Act compliant; field-level PHI encryption
- BAAs executed with every vendor in our stack
- Append-only audit trail on every PHI access event
- Lawyer-reviewed MSA and Referral Partner Agreement templates

DIRECT LINE



Naitik Joshi

CO-FOUNDER, CEO & CTO · HALCYON MEDICAL BILLING LLP

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Replies received before 27 May 2026 will be confirmed within one business day. A signed referral agreement template is available on request.